



Capital
Markets

June 4, 2026

Vertex Pharmaceuticals Incorporated

Navigating the Catalyst Path: See Mix Of Potential NT Clearing Events and Net Favorable Setups

Our view: VRTX shares have traded off considerably of late and are now near YTD lows, giving back much of the gain seen following the highly-encouraging povetacicept ph.III interim IgAN data. We believe this pullback is largely a function of investor trepidation around a near-term catalyst path that - on the surface - may appear to skew toward risk; however, we think the setup for the next several catalysts is considerably more favorable than the market appears to be pricing in, and we take this note as an opportunity to lay out our thinking heading into what we believe are important data readouts this summer and later this year/early-'27. We remain highly encouraged by VRTX's prospects across its CF franchise and growing immunology pipeline, and reiterate our Outperform rating and \$543 price target.

Key points:

Key Catalyst 1: Sionna's Summer Ph.IIa Readout for SION-719 in Combo with Trikafta

Background. Sionna (SION) (which we also cover) is conducting its PreciSION ph.IIa crossover study evaluating NBD1 stabilizer SION-719 as an add-on to standard-of-care Trikafta in ~16 F508del homozygote adult CF patients; the trial is assessing safety, PK, and the CFTR biomarker sweat chloride (SwCl), with data expected in the summer. We believe a meaningful portion of recent weakness in VRTX shares can be attributed to investor concern that a positive SwCl readout for '719 on top of Trikafta would validate the predictiveness of SION's preclinical HBE assay and that their proprietary dual combinations with '451 paired with either '2222 or '109 could ultimately credibly threaten VRTX's CF dominance.

However, we continue to see several reasons why the risk to VRTX may be limited:

1. **HBE assay translatability outside of VRTX has been historically poor** with differences in cell physiology, drug PK/biodistribution, and assay equipment and procedures contributing to variable results even across similarly designed assays. Importantly, we believe that SION uses different cells and somewhat different conditions vs. VRTX's historically predictive model (based on available preclinical information in review documents for Trikafta/Alyftrek). Additionally, our analyses have also suggested inter-assay variability among SION's own experiments based on the relative concentrations of ETI used, and Trikafta performs differently in SION's assays - all underscoring that these assays, while informative, can have limited translational reliability.

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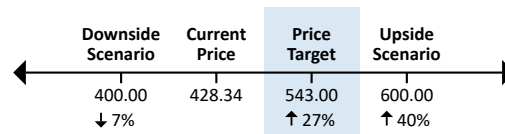
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Outperform

NASDAQ: VRTX; USD 428.34

Price Target USD 543.00

Scenario Analysis*



*Implied Total Returns

Key Statistics

Shares O/S (MM):	259.3	Market Cap (MM):	111,069
Dividend:	0.00	Yield:	0.0%
		Avg. Daily Volume:	1,211,683

RBC Estimates

FY Dec	2025A	2026E	2027E	2028E
EPS, Ops Diluted	18.40	18.80	18.77	21.50
P/E	23.3x	22.8x	22.8x	19.9x
Revenue	12,001.3	13,004.5	13,610.0	14,650.6
EPS, Ops Diluted	Q1	Q2	Q3	Q4
2025	4.06A	4.52A	4.80A	5.03A
2026	4.47A	4.74E	4.77E	4.83E
Revenue				
2025	2,770.2A	2,964.7A	3,076.4A	3,190.0A
2026	2,986.9A	3,266.9E	3,325.6E	3,426.0E

Revenue: \$ in millions

All values in USD unless otherwise noted.

Priced as of prior trading day's market close, EST (unless otherwise noted).

2. Even if SwCl benefits are demonstrated, the Alyftrek experience tells us this does not necessarily translate to FEV1 benefit. The most important regulatory and commercial endpoint in CF is lung function - and data from VRTX's Alyftrek demonstrated that additional SwCl improvement over Trikafta did not result in any FEV1 benefits. Our KOL feedback has suggested that this might have been due to the standard of care approaching an efficacy ceiling and so additional SwCl improvements may not translate to meaningful function improvements, unless SwCl improvements over SoC is ~20mmol. We acknowledge, though, that Alyftrek data were largely in a heterozygous population vs. the homozygous population SION is studying, and the possibility that for a drug with a different mechanism from Trikafta (which Alyftrek does not have), sweat chloride translation to FEV-1 could be stronger (SION views a 10mmol SwCl change as potentially resulting in a 3% improvement in FEV1).

3. Even if '719 were de-risked, this does not necessarily de-risk '451. As SION's own management has indicated, '719 was selected for the add-on study in part because of its higher potency relative to '451 ('451 has higher exposure), but this could then potentially limit the predictable translatability of any SwCl benefits from '719 to '451, the drug they would ultimately move forward in a combination that could theoretically compete for market share with VRTX.

4. Even under the most bullish scenario for SION, the competitive threat to VRTX is years away, and pivotal trials would likely need to be vs. current SoC Alyftrek. Even if '719 data look compelling and SION advances '451 combinations into a ph.II and subsequently a ph.III, they would need to demonstrate clearly superior benefit versus Trikafta and/or Alyftrek - a very high bar given VRTX's market entrenchment, 2x/day dosing of SION's agents, and the fact that any SION approval would realistically not come before 2030 at the earliest.

We see several scenarios and stock reactions depending on SION's data:

- 1. SwCl <10mmol (70% likelihood, >10%):** We think that in the case that '719 does not meet the company bar of 10mmol change in SwCl, that VRTX shares may trade materially higher and back towards ~\$500/share (+15%) - where shares were trading earlier this year post pove topline.
- 2. SwCl ~10mmol (15% likelihood, +5-10%):** In the event that SION meets their SwCl bar, we think that VRTX shares may trade up 5-10%, as a key overhang would be lifted, there would be uncertainty of translation to '451 given potency differences with '719, and uncertainty about whether the SwCl benefits might be enough to translate to FEV1 benefits.
- 3. SwCl >10mmol (10% likelihood, -5%):** We believe that if SION is able to demonstrate SwCl benefits in the low-teens that this could potentially start to de-risk the opportunity for '451 more meaningfully, though other questions about translation from '719 and FEV1 benefits would still remain. We would not be shocked to see VRTX shares open down then recover, especially if '719's SwCl change was no more than the low to mid teens.
- 4. SwCl >>10mmol (5% likelihood, -10%):** In the case that '719 demonstrates much greater SwCl benefits on top of Trikafta, we think that '451 could become more materially de-risked for investors, and could see the likelihood SION's NBD1 stabilizers could demonstrate FEV1 benefit increase.

Key Catalyst 2: Otsuka's Sibeprenlimab (Voyxact) Ph.III eGFR Data (June 5, 2026)

Background. Otsuka has guided to reporting topline 2-year eGFR from its pivotal ph.III sibeprenlimab (Voyxact) study between June and August 2026, which is required for full approval, and an abstract for the a "prespecified interim analysis" of eGFR is slated for presentation on June 5 at the ERA Congress. Recall that Voyxact (an APRIL inhibitor) achieved ~51% placebo-adjusted proteinuria reductions at 9 months in its ph.III - closely in line with pove's ~50% pbo-adjusted reduction at 36 weeks in the interim RAINIER cohort. Commercially, Voyxact

has launched well with cumulative total new patient starts exceeding 1,000 since launch and ~¥4.7B (~\$29M) in 1Q26 sales, with FY2026 guidance of ~¥25B (~\$156M).

Why this matters for VRTX. Given the similar proteinuria profiles between sibeprenlimab and povetacicept, the sibe eGFR readout has meaningful and direct readthrough implications for pove's own ph.III eGFR endpoint, which we expect to read out around early 2028 - and while we do not believe there is major risk, any hiccup could raise questions about the class and about this key value-driving pipeline asset for VRTX (so some may be on the sidelines ahead of these data). Our extensive KOL diligence ([here](#), [here](#), and [here](#)) has consistently suggested that sustained proteinuria reductions of the magnitude demonstrated across APRIL and APRIL/BAFF inhibitors will likely translate to meaningful eGFR benefits in IgAN (slower decline vs. placebo or preservation/stabilization of eGFR); pove showed eGFR stabilization and even improvement vs. baseline in the ph.I/II RUBY-3 study, atacicept ph.IIb eGFR showed stabilization with longer-term follow-up, and the recently-published telitacicept ph.III China data demonstrated relative eGFR stabilization (-1%) vs. placebo decline (-8%) within 39 weeks. In fact, during our recent conversation with the company, VRTX mgmt's strong belief on proteinuria reductions translating to eGFR stabilization in IgAN suggests to us they are unlikely to be seeing any surprises emerging from ongoing ph.III follow-up, and VERA's recent alignment with FDA on an earlier eGFR analysis (which VRTX could potentially do as well) further supports both the kidney KOL community and regulators' comfort with the class's potential to provide meaningful, and potentially early, eGFR stabilization signals.

We see several scenarios and stock reactions:

- 1. Sibe demonstrates eGFR stabilization (30% likelihood, +5-7%):** We believe that if sibeprenlimab is able to demonstrate stabilization of eGFR that this would likely lead to full approval of the drug, and we would see strong readthroughs to VRTX's pove, which has demonstrated similar improvements in proteinuria. However, we note that since ~8% of sibe treated patients in its pivotal trial had neutralizing ADAs, which impacted proteinuria reductions, and so we could see eGFR curves modestly impacted.
- 2. Sibe demonstrates slowing decline in eGFR vs. placebo (60% likelihood, +3-5%):** We believe that this scenario is the most likely for sibe (taking into account likely impact of neutralizing ADAs), but since pove has not demonstrated any such neutralizing ADAs, we believe that pove could potentially demonstrate even better eGFR outcomes relative to sibe, representing a meaningful clinical and commercial differentiator, which could result in additional upside.
- 3. Sibe demonstrates no material improvement in eGFR vs. placebo (10% likelihood, -10%):** We see a low likelihood of sibe not demonstrating any eGFR improvements vs. placebo given our KOL feedback and other historical trials, and in this scenario, we could see VRTX shares trade down meaningfully, as it would call into uncertainty their accelerated approval pathway, though neutralizing antibodies could be an explanation as to why sibe's proteinuria reductions might not translate to eGFR benefits as much as pove's could.

Key Catalyst 3: Ph.IIb/III pMN Data from OLYMPUS (2H26)

Background. The OLYMPUS trial is a ph.IIb/III adaptive, randomized, active-controlled study evaluating povetacicept versus tacrolimus (a calcineurin inhibitor) in primary membranous nephropathy (pMN); the study is set to enroll a total of 176 patients, with the primary endpoint the proportion of participants achieving complete clinical remission at Week 104. VRTX recently completed enrollment in the ph.IIb portion, which was measuring the change in anti-PLA2R autoantibodies from baseline through 12 wks in order to identify the dose of pove to take forward into ph.III portion, which the company also recently initiated; we anticipate a potential ph.IIb readout in 2H26. RUBY-3 data remain the strongest reference point for how pove could perform in this indication, where at 48 weeks, pove demonstrated an impressive 100% serologic remission (anti-PLA2R negativity), 82% reduction in UPCR in 5 pMN patients, 40% complete

clinical remission (defined as UPCR <0.5 g/g), and stable eGFR (-0.3 mL/min/1.73m²). Although VRTX has not disclosed details around when the ph.IIb data might be disclosed, we believe that there could be a chance to see the data later on in the year at a major medical meeting - potentially ASN, October 21-25.

Potential stock reaction (+3-5%/-2-3%): We think that if the ph.IIb continues to demonstrate high rates of serologic remission, which our KOL feedback has suggested is important indicator for disease control, we could see notable upside (+3-5%) as this could be de-risking for ph.III trial, though if efficacy is limited (<80% serologic remission), hypogammaglobulinemia/infection risks are high, or other safety signals appear, we expect to see modest downside (-2-3%) as pove would likely not be competitive vs. other B-cell depleting MoA drugs - though we believe this would be limited, as we view this as a smaller potential opportunity than IgAN (est. \$850M, similar to consensus, vs. \$3.5B IgAN), and have a conservative PoS (30%) in our model.

Key Catalyst 4: VX-828 Sweat Chloride Data (2H26) - CF Next-Gen 3.0 Corrector POC

Background. VRTX is on track to share SwCl and safety data from the CF patient cohort of the VX-828 proof-of-concept ph.I study in 2H26. While normally, data from a ph.I trial is not likely to move the needle for VRTX, we believe that '828 could potentially set a perceived higher bar for SwCl benefits, which could help to further deepen the competitive moat when considering upcoming SION data (data from other next-generation 3.0 CFTR correctors, VX-581 and VX-272, serve as additional shots on goal though more meaningful data from these assets in CF pts likely would not be until sometime next year). Recent updates to the ct.gov listing suggests that enrollment is complete, with final enrollment 165 pts, fewer than the original 255 pts expected, but understanding the rationale for this change is challenging as this is a multi-faceted study including SAD/MAD in HVs, a DDI component, and various dual and triple combos in both HVs and het/min CF pts - it could be due to better-than-expected results, a safety signal or poor efficacy, or an attenuation of (or an unfavorable signal in) the optional DDI component. The trial remains active and primary completion date was updated to mid-July, suggesting that we could see data sometime soon after - possibly August or September.

Potential stock reaction (+3%/-2%): Alyftrek has set a high bar, with 90% of CF patients achieving sweat chloride below 60 mmol/L and two-thirds reaching the "normal" range below 30 mmol/L on a once-daily regimen, and so mgmt has previously indicated that this next-generation program would likely need to demonstrate meaningful SwCl improvements to advance. The impact to VRTX from this readout is likely to depend on the cadence of data from SION's '719 - if competitor data comes ahead of '828, we think that '828 could become a more important event depending on if '719 demonstrated meaningful SwCl changes on top of Trikafta; however, if '828 data comes ahead of '719 readout, there is the potential for VRTX to set an even higher bar for '828 (+3%), helping to potentially mitigate downside from better than expected SION data, though we could see an outright failure of '828 somewhat disappoint (-2%). That all said, we do note key differences in study design - VRTX is testing '828 in a dual alongside d-ivacaftor and triple alongside d-iva+tez in het/min pts x28d, whereas SION is testing '719 in a quad add-on on top of iva/tez/elex run-in in F508del homozygous pts x14d - will preclude head to head comparisons, so we would more be looking for whether sweat chloride changes from baseline from '828/d-iva +/- tez exceed the ~50mmol/L reductions seen for Alyftrek in ph.II at that timepoint and as such signify a larger competitive moat.

Key Catalyst 5: Inaxaplin AMPLIFIED Data in Diabetic/Moderate Proteinuria Population (2H26)

Background. AMPLIFIED is a ph.IIb open-label study of inaxaplin in AMKD patients with more modest proteinuria (vs. those in the ongoing ph.III AMPLITUDE study) or who have diabetes

- subsets of considerable commercial interest, particularly in the context of recent data from MAZE that has elevated investor attention - with data expected in 2H26.

Potential stock reaction (+3-7%/-3-5%): We view AMPLIFIED data as an incremental de-risking event that could provide additional validation of inaxaplin's proteinuria activity across a broader and commercially relevant patient population ahead of the main ph.III interim early-'27, though believe it is important to note these are likely more challenging populations to show as robust an effect in. Recall that data from competitor MAZE, which is developing another drug for AMKD, had recently demonstrated 61.8% uACR reduction in non-diabetic AMKD FSGS patients (n=4), vs. VRTX's earlier 47.6% uPCR reduction from a ph.II in a similar population (n=13), though we note that MAZE's data set was highly variable and sample sizes small. Importantly though, we estimate that in the diabetic AMKD population, we back-calculate that MAZE's drug likely produced only a ~15-20% reduction in proteinuria, highlighting potential challenges in this population - who likely has multi-factorial influences for their kidney dysfunction. A caveat to comparing across the studies is that MAZE included pts with uACR of 300-3,000 mg/g, whereas we believe AMPLIFIED is enrolling pts with more modest baseline proteinuria (100-400 mg/g). This difference can potentially cut both ways: on one hand, patients with less advanced/complex disease may have more response to an ApoL1 modulator, but on the other hand, a lower baseline starting uACR - applicable to both the diabetics and non-diabetics - could leave less dynamic range to show major improvements. While we believe better signals in the diabetic population in VRTX's study could potentially result in meaningful stock upside (+3-7%), as the diabetic subset represents a sizable proportion of the real-world AMKD population (+100K pts, as per company, on top of 150K AMPLITUDE population), and this reignites enthusiasm around inaxaplin's likelihood of success, breadth of opportunity, and competitive positioning. However, we could see some initial downside (3-5%) if data look comparable to or less robust than MAZE (VRTX has implied they would like to see ~30% proteinuria reductions) and this generates concerns around inaxaplin's efficacy extending beyond FSGS in AMPLITUDE (even though we believe fundamental readthroughs to AMPLITUDE should be limited).

Early 2027 Catalysts: Inaxaplin Ph.III Interim (AMKD) and Suzetrigine DPN Ph.III Data (Early '27)

Inaxaplin Ph.III 48-Week Interim

Background: Enrollment in the 48-week interim analysis of the AMKD ph.III study had completed in 2H25 with early-'27 readout representing the next major binary event for the kidney franchise. Recall the statistical design of the interim tests both proteinuria reductions and slope differences between the inaxaplin and placebo arms in eGFR trajectory. Proteinuria reductions should be robust, in our view, as the ph.II had shown 48% uPCR reductions over only 13 weeks, effects were consistent across patients, and even allowing for some degradation in going beyond just FSGS pts (MAZE data showed ~20% lesser proteinuria reductions for the fuller non-diabetic group as compared to the FSGS subset), we still believe proteinuria reductions could come in at the 40-50% range. Translation to eGFR is a bit more complicated, though, as given the heterogeneity of the population (and limited established natural history data), our KOL feedback has consistently been that longer follow-up (at least 2 years) may be needed to robustly observe an eGFR slope separation - though VRTX believes enrolled the population would have 3-8mL/min reductions over one year so demonstrating near-stabilization on inaxaplin (which the short, small ph.II did show for 13 wks) would be very discernible.

Potential stock reaction (+8-10%/-5%): If the 2-year data - likely available in early to mid-2028 - demonstrate clear eGFR slope divergence between the treatment and placebo arms, this would meaningfully de-risk what we estimate to be a ~\$3.3B WW out-year opportunity in a disease with no currently approved treatments (est. +8-10%, noting that increasing PoS to 100% would increase our DCF-based fair value by +9%). Our KOLs indicate they would use

inaxaplin in all eligible AMKD patients given the lack of alternatives, which speaks to the depth of unmet need. We believe the stock has already largely discounted a non-accelerated approval outcome at the 48-week interim, limiting downside risk from an interim that demonstrates only proteinuria improvements but no eGFR differences yet (-2-3%), and the path to full approval on a 2-year eGFR endpoint remains well-supported by our KOL conversations and the precedent set by sparsentan in FSGS. More modest (<25-30%) proteinuria reductions without eGFR slope differences could call into greater question the potential of the program, and we believe lead to 5% stock downside (removing from our model would decrease our DCF-based fair value by 6%).

Suzetrigine DPN Ph.III

Background: Two ph.III DPN studies for suzetrigine are expected to complete enrollment by year-end 2026, with topline data anticipated in early 2027, though these trials remain higher risk given prior lumbosacral radiculopathy (LSR) ph.II failure - the only placebo-controlled chronic pain data for suzetrigine.

Potential stock reaction (+7-10%/-3-5%): We believe that at current stock prices that any expectations for chronic pain are washed out (we receive few investor inbound on the pain program), and so despite the risks, we think the stock setup into the readout is considerably improved. We continue to see meaningful risk (est. 40% PoS). However, KOLs we have spoken with have been less skeptical than we have, noting LSR is a heterogeneous and difficult-to-study indication and their belief that the similar pain reductions seen in DPN with suzetrigine vs. pregabalin may be suggestive of real activity. Additionally, based on our KOL checks, DPN may be a generally less centrally mediated pain syndrome relative to LSR, driven more by peripheral nociceptive input - precisely the domain where Nav1.8 is highly expressed and where suzetrigine's mechanism is most relevant. We think there is modest downside risk should the trial not succeed (-3-5%) though if suzetrigine demonstrates efficacy in DPN, we think upside would be substantial (+7-10%); even assuming it would be limited to DPN rather than broad neuropathic pain states, we still believe that would be a \$1.5-2B market opportunity.



Exhibit 1 - VRTX Income Statement

	2026														
(\$ in thousands except per share ite	2025A	1Q26A	2Q26E	3Q26E	4Q26E	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E
Trikafta/Kaftrio	10,314,921	2,354,700	2,474,462	2,424,183	2,385,524	9,638,734	8,968,354	8,366,215	7,817,030	7,339,656	6,991,915	6,791,332	6,743,918	6,628,776	6,520,891
U.S.	6,470,117	1,381,455	1,518,758	1,485,830	1,475,860	5,861,904	5,544,176	5,141,727	4,828,470	4,508,310	4,328,835	4,135,177	4,097,664	3,980,525	3,870,895
ex-U.S.	3,844,803	973,109	955,704	938,352	909,664	3,776,830	3,424,178	3,224,487	2,988,559	2,831,346	2,663,080	2,656,155	2,646,254	2,648,251	2,650,195
Alyftrek	838,894	424,400	552,364	633,787	740,663	2,353,325	3,443,952	4,296,509	5,071,364	5,763,125	6,261,798	6,591,201	6,759,100	6,994,852	7,222,117
U.S.	754,456	316,409	413,041	464,915	530,061	1,724,426	2,371,095	2,951,423	3,422,390	3,893,682	4,207,592	4,522,749	4,678,437	4,914,206	5,141,514
ex-U.S.	84,439	110,103	139,323	168,872	210,602	628,900	1,072,857	1,345,086	1,648,974	1,869,442	2,054,207	2,068,452	2,080,663	2,080,646	2,080,603
Symdeko/Symkevi	158,449	33,066	32,146	30,548	29,823	125,583	87,296	87,247	86,817	86,330	86,322	86,372	17,115	9,648	9,833
U.S.	54,274	10,230	11,578	11,777	11,879	45,465	8,412	8,578	8,753	8,931	9,112	9,287	9,466	9,648	9,833
ex-U.S.	104,175	22,836	20,568	18,771	17,943	80,118	78,884	78,669	78,064	77,399	77,209	77,085	7,649	-	-
Orkambi	220,424	42,248	40,312	37,921	36,199	156,680	139,870	120,565	104,554	9,826	1,500	1,103	747	435	170
U.S.	17,531	3,348	4,022	4,036	4,228	15,634	15,536	13,589	11,782	1,933	1,500	1,103	747	435	170
ex-U.S.	202,893	38,900	36,290	33,885	31,971	141,046	124,334	106,976	92,771	7,893	-	-	-	-	-
Kalydeco	321,651	58,324	58,461	56,495	57,106	230,387	22,811	6,847	6,907	6,966	7,025	7,072	7,119	7,165	7,210
U.S.	118,390	14,515	14,782	12,891	13,298	55,487	6,789	6,847	6,907	6,966	7,025	7,072	7,119	7,165	7,210
ex-U.S.	203,261	43,809	43,679	43,603	43,808	174,900	16,022	-	-	-	-	-	-	-	-
US CF Sales	7,414,768	1,725,958	1,962,181	1,979,450	2,035,326	7,702,916	7,946,009	8,122,166	8,278,303	8,419,823	8,554,064	8,675,389	8,793,433	8,911,979	9,029,422
ex-US CF Sales	4,439,571	1,188,757	1,195,564	1,203,484	1,213,988	4,801,794	4,716,275	4,755,218	4,808,368	4,786,081	4,794,496	4,801,691	4,734,566	4,728,897	4,730,798
Casgevvy	114,789	42,900	67,396	81,150	92,280	283,023	365,282	442,474	510,074	569,704	650,056	717,308	777,345	824,137	885,314
U.S.	97,228	31,312	51,167	62,666	67,139	212,284	275,854	334,563	384,451	428,241	494,592	539,150	584,178	622,703	667,676
ex-U.S.	17,561	10,886	16,229	18,484	25,141	70,739	89,427	107,911	125,623	141,463	155,463	178,158	193,167	201,433	217,638
Journavx in pain (risk-adjusted)	59,668	29,000	41,749	61,538	84,388	216,768	325,971	418,812	545,112	715,248	908,626	1,102,882	1,276,043	1,434,700	1,593,538
U.S.	59,668	29,094	41,749	61,538	84,388	216,768	325,971	401,044	513,546	639,499	778,304	909,104	1,022,567	1,132,363	1,243,677
ex-U.S.	-	-	-	-	-	-	-	17,768	31,567	75,748	130,322	193,779	253,476	302,337	349,861
VX-880 in T1D (risk-adjusted)	-	-	-	-	-	-	94,967	264,504	442,192	601,502	757,638	867,585	957,865	1,049,451	1,127,299
U.S.	-	-	-	-	-	-	94,967	221,844	341,775	445,265	552,441	611,614	675,651	741,596	793,465
ex-U.S.	-	-	-	-	-	-	-	42,660	100,417	156,238	205,197	255,971	282,215	307,855	333,833
Inaxaplin in AMKD (risk-adjusted)	-	-	-	-	-	-	22,681	116,484	279,199	452,579	638,037	831,107	1,033,982	1,201,551	1,355,782
U.S.	-	-	-	-	-	-	22,681	109,203	240,190	369,905	514,573	666,958	827,376	950,656	1,070,456
ex-U.S.	-	-	-	-	-	-	-	7,282	39,009	82,674	123,465	164,149	206,606	250,895	285,325
Povetacept (risk-adjusted)	-	-	-	-	-	138,804	530,905	1,033,610	1,674,298	2,475,790	3,234,754	3,868,758	4,511,661	5,119,673	5,119,673
U.S.	-	-	-	-	-	138,804	442,824	752,681	1,205,428	1,736,077	2,203,840	2,598,207	2,999,510	3,410,509	3,410,509
ex-U.S.	-	-	-	-	-	-	88,081	280,929	468,870	739,713	1,030,914	1,270,550	1,512,150	1,709,163	1,709,163
Product revenues, net	11,980,600	2,986,900	3,266,891	3,325,621	3,425,983	13,004,501	13,609,988	14,650,564	15,896,859	17,219,234	18,778,706	20,230,716	21,441,992	22,662,375	23,841,825
Royalty revenues	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Collaborative revenues	20,700	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Total revenues, net	12,001,300	2,986,900	3,266,891	3,325,621	3,425,983	13,004,501	13,609,988	14,650,564	15,896,859	17,219,234	18,778,706	20,230,716	21,441,992	22,662,375	23,841,825
Cost of product revenues	1,620,000	384,600	454,408	460,208	469,505	1,768,721	1,835,783	1,936,766	2,056,642	2,174,318	2,333,610	2,481,872	2,607,688	2,711,342	2,835,942
Costs related to CRSP milestone amortization	20,000	5,000	5,000	5,000	5,000	20,000	20,000	20,000	20,000	20,000	20,000	20,000	20,000	20,000	20,000
Royalty expense	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
R&D	3,491,500	859,300	902,265	947,378	994,747	3,703,690	3,888,875	4,005,541	4,045,597	4,086,053	4,126,913	4,168,182	4,209,864	4,251,963	4,294,482
IPR&D	133,000	500	50,000	25,000	25,000	100,500	100,500	100,500	100,500	100,500	100,500	100,500	100,500	100,500	100,500
SG&A	1,493,700	432,200	458,132	481,039	505,091	1,876,461	1,970,284	2,039,244	2,100,421	2,163,434	2,228,337	2,295,187	2,364,043	2,434,964	2,508,013
Total operating expenses	6,738,200	1,676,600	1,864,805	1,913,625	1,994,343	7,449,373	7,795,442	8,082,051	8,303,160	8,524,305	8,789,361	9,045,742	9,282,095	9,498,769	9,738,938
Income from operations	5,263,100	1,310,300	1,402,086	1,411,996	1,431,640	5,555,128	5,814,546	6,568,512	7,593,699	8,694,929	9,989,346	11,184,974	12,159,897	13,163,607	14,102,887
Interest income, net	477,600	114,800	115,661	116,528	117,402	464,392	195,140	308,861	396,193	522,553	670,069	808,635	959,761	1,119,089	1,279,757
Other (expense) income, net	(2,700)	2,000	2,000	2,000	2,000	8,000	8,400	8,820	9,261	9,724	10,210	10,721	11,257	11,820	12,411
Net pre-tax income	5,738,000	1,427,100	1,519,747	1,530,525	1,551,042	6,027,520	6,018,087	6,886,193	7,999,153	9,227,206	10,669,625	12,004,330	13,130,914	14,294,515	15,395,055
Provision for income taxes	991,000	280,100	303,949	306,105	310,208	1,200,363	1,203,617	1,377,239	1,599,831	1,845,441	2,133,925	2,400,866	2,626,183	2,858,903	3,079,011
Net income (non-GAAP)	4,747,000	1,147,000	1,215,798	1,224,420	1,240,834	4,827,157	4,814,469	5,508,955	6,399,322	7,381,765	8,535,700	9,603,464	10,504,731	11,435,612	12,316,044
Income attributable to noncontrolling in	-	-	-	-	-	-	-	-	-	-	-	-	-	-	-
Net income to VRTX (non-GAAP)	4,747,000	1,147,000	1,215,798	1,224,420	1,240,834	4,827,157	4,814,469	5,508,955	6,399,322	7,381,765	8,535,700	9,603,464	10,504,731	11,435,612	12,316,044
Earnings per share (non-GAAP)	\$18.40	\$4.47	\$4.74	\$4.77	\$4.83	\$18.80	\$18.77	\$21.50	\$25.01	\$28.89	\$33.45	\$37.68	\$41.27	\$44.99	\$48.26
Shares Outstanding (Basic)	255,800	254,100	254,200	254,300	254,400	254,250	254,060	253,720	253,380	253,040	252,700	252,360	252,020	251,680	252,680
Shares Outstanding (Diluted)	258,025	256,300	256,700	256,800	256,900	256,750	256,560	256,220	255,880	255,540	255,200	254,860	254,520	254,180	255,180

Source: Company reports and RBC Capital Markets estimates



Exhibit 2 - VRTX Potential Milestones and Pipeline

Milestones		
Product	Event	Timeline
Journavx (suzetrigine)	Data from two ph.III DPN studies	Early-'27
Alyftrek	Global regulatory submissions in pediatric pts 2-5 yrs	1H26
	Data from pediatric pts 1-2 yrs	2026+
Next-gen combo (VX-828, VX-118, tezacaftor)	Data from combo in CF pts	2026
	Potential accelerated approval in IgAN	2H26
Povetacicept	Data from ph.II/III pMN pivotal study	2H26+
	Data from ph.II gMG study	2H26+
	eGFR data from Otsuka siveprentlimab	Jun-Aug, 2026
VX-993	Data from ph.II study in DPN	2027
Casgevy	Potential U.S. approval for SCD/TDT pts 5-11 yrs of age	2H26
	Readout from ph.III trial in APOL1 mediated kidney disease	Early-'27
Inaxaplin (VX-147)	Potential accelerated approval filing in AMKD	2027
	Data from moderate proteinuria or diabetes population	2H26
VX-670	Completion of enrollment and dosing in DM1 study	Mid-'26
	DM1 study ph.I/II data	2H26
VX-828	SwCl data from ph.I study	2H26
VX-407	Ph. II ADPKD AGLOW study data	2027
Zimislecel	Data from pivotal study	2026+
	Resume dosing post manufacturing review	2026
DMD gene editing therapy	IND filing following additional work	2026+

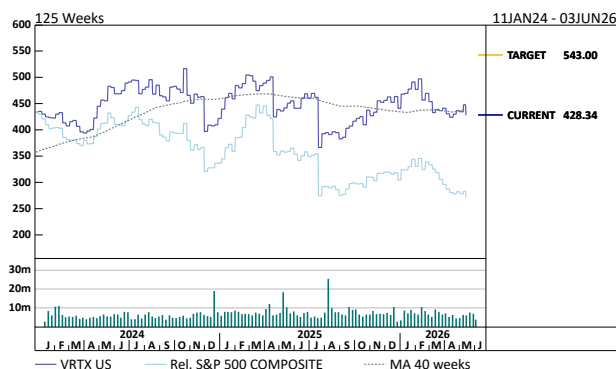
Pipeline		
Drug/Target (Partner)		
Kalydeco	Cystic fibrosis gating, residual function mutations	Marketed
Orkambi	Cystic fibrosis F508del homozygous	Marketed
Symdeko/Symkevi (tez/iva)	Cystic fibrosis F508del homozygous	Marketed
Trikafta/Kaftrio (elexacaftor/tez/iva)	Triple combination for F508del homozygous and het/min CF pts	Marketed
Casgevy (CRISPR)	Beta-thalassemia, sickle cell disease	Marketed
Alyftrek (VX-121/tezacaftor/VX-561)	Cystic fibrosis for F508del homozygous and het/min CF pts	Marketed
Journavx (suzetrigine, NaV1.8 inhibitor)	Acute pain, diabetic peripheral neuropathy, lumbosacral radiculopathy	Marketed, Phase III
Inaxaplin (VX-147)	APOL1-targeted small molecule for FSGS	Phase III
Povetacicept	IgAN, gMG, wAIHA, pMN	Phase II/III
Zimislecel (VX-880, naked cell therapy)	Severe type 1 diabetes (T1D) or T1D with kidney transplant	Phase II/III
VX-150	Pain	Phase II
VX-970 (M6620; Merck KGaA)	Solid tumors	Phase II
VX-993 (NaV1.8 inhibitor)	Oral and IV formulation	Phase II
VX-973 (NaV1.8 inhibitor)	Oral formulation	Phase II ready
VX-670 (oligonucleotide)	DM1	Phase I/II
Next-gen 3.0 CFTR ('828, '581, '272)	CF	Phase I
AAT corrector molecules	Alpha-1 Antitrypsin Deficiency (AAT)	Preclinical
CRISPR/Exonics	Gene editing for DMD and DM1	Preclinical
Hypoimmune cell	Type 1 diabetes (T1D)	Preclinical
Kymera Therapeutics partnership	Small molecule protein degraders for multiple diseases	Preclinical
Ribometrix, Inc. partnership	RNA-targeted small molecules	Preclinical
Affinia Therapeutics partnership	AAV capsids for genetic therapies	Discovery
Skyhawk Therapeutics partnership	Small molecule RNA splicing modulators	Discovery
CF corrector molecules	Cystic fibrosis	Discovery
CF CRISPR/Cas9	Cystic fibrosis	Discovery
Small molecules	Sickle cell disease, beta thalassemia	Discovery

Source: Company reports and RBC Capital Markets estimates



Target/Upside/Downside Scenarios

Vertex Pharmaceuticals Incorporated



Valuation

Our \$543 price target is derived from a DCF analysis of the base business, with a 9.0% discount rate and a 2.5% terminal growth rate. Our price target supports our Outperform rating.

Upside scenario

Successful expansion of Kaftrio into Europe, better-than-expected ex-U.S. reimbursement and pricing power, and ex-CF pipeline successes including suzetrigine, inaxaplin, povetacicept, and zimislecel/'264 would lead to upside to our estimates. In such an upside scenario, a potential DCF-based valuation would be \$600.

Downside scenario

Emergence of competition reducing long-term market share, emergence of a safety signal for suzetrigine, povetacicept, inaxaplin, or zimislecel/'264, and eroding pricing power would lead to downside to our estimates. In such a downside scenario, a potential DCF-based valuation would be \$400.

Investment summary

VRTX remains in a strong commercial position with a CF franchise generating solid revenues for at least the next decade, and with little other competition aside from SION's combo approach in CF, which we see as unlikely to materially impact VRTX, we do not believe shares at current levels adequately reflect the potential upside from pipeline successes with VRTX's kidney programs, expansion of pove into autoimmune diseases like IgAN and gMG, and significant optionality should suzetrigine prove efficacious in chronic pain (though this remains higher risk). We believe that given VRTX's strong core revenue, a pipeline that is increasingly becoming more mature, and with optionality to further diversify pipeline with external BD, shares should command a higher valuation.

Key attributes: (1) leader in high-growth, high-margin CF space led by Kalydeco, Orkambi, Symdeko/Symkevi, Trikafta/Kaftrio, and Alyftrek; (2) Alyftrek launch could improve out-year operating margins; (3) high bar for potential future competitors; and (4) pipeline promise demonstrated by povetacicept, and inaxaplin.

Potential catalysts: (1) Potential accelerated approval for povetacicept in IgAN (2H26); (2) Competitor CF data (mid-'26); (3) Early next-gen CF data (2H26), (4) inaxaplin interim data (early-'27).

Risks to rating and price target

Risks include lower-than-expected commercial uptake of Alyftrek, pricing pressure in CF, competition, and emergence of safety issues from pipeline programs like povetacicept, suzetrigine, or inaxaplin.



Key fundamental questions

Our view

Can the cystic fibrosis franchise continue to grow?

Vertex's core commercial cystic fibrosis franchise booked nearly \$12B in revenue in 2025 and likely remains on track to book at least \$12.5B in 2026. The recent Alyftrek launch could expand margins due to lower royalties, and continued growth in US and ex-US markets among younger CF patients and patients with different mutation subsets, we continue to see potential for this franchise alone to grow to \$13B+ in out-years.

How durable of a floor valuation does the core cystic fibrosis franchise set?

There remains limited near- or medium-term competition in the development pipeline, enabling Vertex to dominate the cystic fibrosis space, and we believe any eventual competitor would need to overcome the high bar set by Vertex's cystic fibrosis therapies and see limited concerns from near term CF data from SION's combo given uncertain translatability of preclinical data to clinical meaningfulness. Coupled with a strong IP position and ongoing internal development to further improve upon Vertex's position, we see a uniquely durable commercial franchise that should set a strong floor valuation, particularly relative to biotech/pharma peers.

Can the company continue to find growth through its internal pipeline portfolio?

We believe Vertex has an underappreciated pipeline portfolio that has a fair chance of driving medium- to long-term growth. This includes multiple programs with potential updates next year such as in genetic kidney diseases, hemoglobinopathies, autoimmune disease, and pain.

Key sustainability questions

This section is intended to highlight key sustainability discussion points relevant to this company, as well as our views on the outlook. Both the questions we highlight and our responses will evolve over time as the dialogue between management, analysts and investors continues to advance. We welcome any feedback on the topics.

Our view

What are the most material sustainability issues facing this company?

Like many commercial-stage biotechs, we believe Vertex continues to look for sustainability solutions, such as implementing green laboratory and manufacturing practices, proper clinical trial conduct including ensuring diverse enrollment of patient populations, data transparency initiatives, promotion of diversity among hires at all levels including the management team, and socially responsible drug pricing and access to therapies for patients.

Does the company integrate sustainability considerations into its strategy?

Vertex has historically been committed to prioritizing sustainability practices, and it often exceeds its own ambitious goals on various sustainability metrics. Senior leadership is visibly engaged in sustainability considerations, and we think that a sustainability-focused culture permeates this rare disease company.

What is diversity like at the board/management level?

Vertex has played a leadership role on this front, being the largest major U.S. biopharma company to have a female CEO. Other positives include that just over half of all global employees and just under half of employees at the VP level or higher are women, and one-third of the workforce and new hires is racially or ethnically diverse. However, only 10–20% of leaders at the VP level or higher are also racially or ethnically diverse. There could also be more women on the leadership team, though we note that the Board of Directors is perhaps more appropriately balanced.

How meaningful are the unmet needs for which the company is developing therapies?

The company is focused on bringing to market therapies that can be game-changing for diseases with high unmet needs such as cystic fibrosis, type 1 diabetes, pain, and rare, genetic kidney diseases, to name just a few. The burden in these disease states is significant.

How transparent are governance factors and board structures at the company?

While typical corporate governance documents particularly for a company of Vertex's size are available online as perhaps expected, Vertex does not appear to go above and beyond; for example, it has not elected to disclose political contributions or payments made to trade associations. VRTX has separate Chairman and CEO positions, which we believe incrementally improves the board's ability to effectively operate. At one point, the company had swiftly investigated and terminated its CFO, a longstanding member of the team, for violations of the company's code of conduct, reaffirming its commitment to diversity, inclusion, and respect, though the exact reasons were not disclosed.



Company description

Vertex Pharmaceuticals is a global biotechnology company focused on the development of small molecule drugs for serious unmet diseases. The company has previously developed drugs that were FDA approved for HIV and Hep C, and it now has multiple approved drugs including Kalydeco, Orkambi, Symdeko, Trikafta/Kaftrio, and Alyftrek for the treatment of various mutations for cystic fibrosis, Journavx for acute pain, and Casgevy in sickle cell disease, and is further developing a pipeline of additional CF, pain, and kidney drugs for other populations that current drugs do not address. Vertex aspires to become a fully integrated biopharmaceutical company from discovery to commercialization and has its own global sales and marketing commercial infrastructure.

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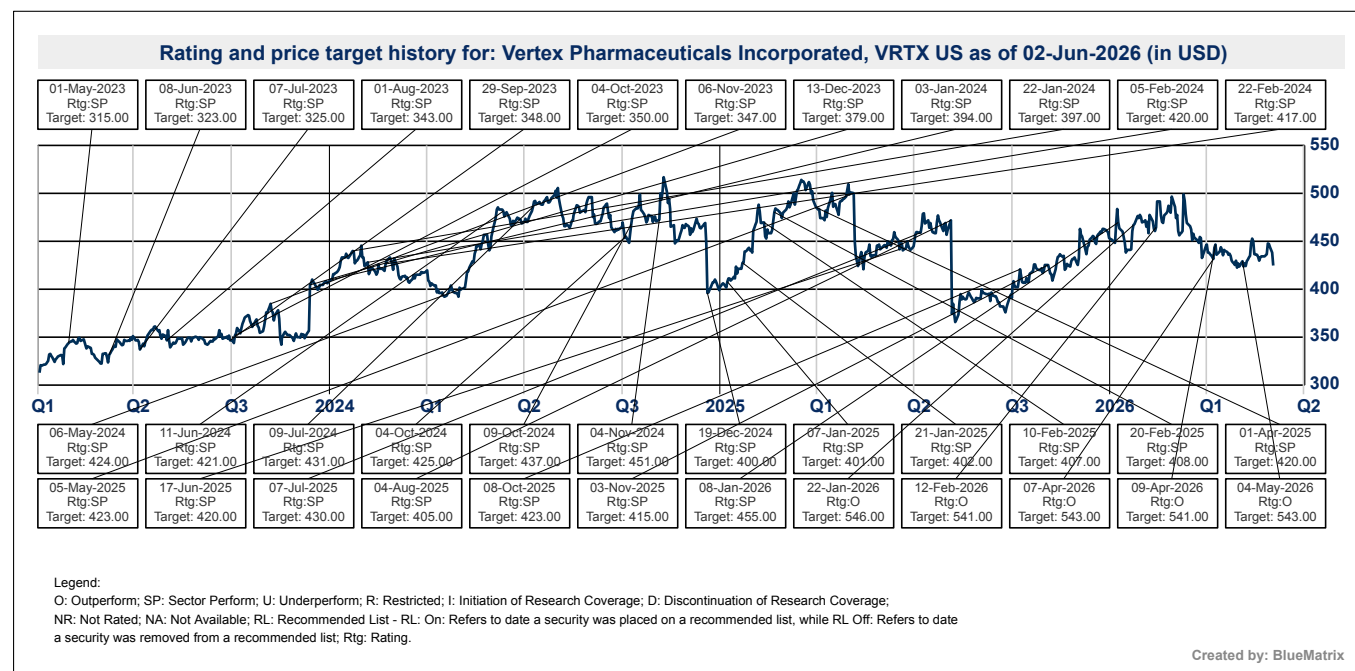
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Rating	Count	Percent	Count	Percent
			Count	Percent
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Vertex Pharmaceuticals Incorporated**Valuation**

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